Fregoli Syndrome

Fregoli Syndrome is a rare psychiatric and neuropsychiatric condition that belongs to the group of delusional misidentification syndromes. It was first described in 1927 by Courbon and Fail. The main characteristic of the syndrome is the belief that different people in the individual’s surroundings are actually the same familiar person who appears before them in different disguises. In other words, the person may recognize that the people they encounter have different appearances; however, they believe that they are actually the same person. Therefore, Fregoli Syndrome is associated with an impairment in the process of distinguishing and identifying one person from other people.

In Fregoli Syndrome, an individual may perceive multiple strangers or different people in their surroundings as a single familiar person. The belief that the familiar person is following them, appearing before them by taking on the appearance of different people, or concealing themselves may occur. In some patients, this misidentification may not be limited to a single person and may involve multiple people. In this respect, Fregoli Syndrome is not simply a matter of “mistaking one person for another”; rather, it involves a strong and firmly held false belief about the identity of the people encountered by the individual.

Fregoli Syndrome may occur on its own or in association with different psychiatric or neurological conditions. In the first source, 52% of the 119 patients studied were reported to be associated with primary psychosis, while 42% were associated with secondary psychosis. Schizophrenia was the most common diagnosis in the primary psychosis group. In secondary cases, neurological diseases, stroke, and traumatic brain injury were particularly notable. Therefore, when Fregoli Syndrome is observed in an individual, not only psychiatric causes but also possible underlying neurological causes should be taken into consideration.

Disorders involving the right hemisphere and frontal regions of the brain are thought to be particularly important in the development of Fregoli Syndrome. In the meta-analysis reported in the first source, right-sided brain lesions were found to be more common than left-sided lesions among patients who underwent neuroimaging. The second source also indicates that lesions of the right hemisphere, particularly the right frontal region, are prominent in delusional misidentification syndromes. Dysfunction in these regions is thought to contribute to misidentification by affecting processes such as familiarity, reality evaluation, and the integration of memories.

Facial recognition and emotional evaluation processes are also important in understanding Fregoli Syndrome. The second source states that abnormal functioning of the fusiform face area in the ventral temporal region, which is associated with facial recognition, and the amygdala, which is associated with emotional regulation, may be related to delusional misidentifications. The third source considers Fregoli Syndrome a form of “hyper-identification” and states that the individual’s ability to distinguish the unique identity information associated with a particular person may be impaired.

Fregoli Syndrome may occur in some cases during a first psychotic episode. According to the findings of the first source, patients with secondary psychosis were more likely to experience Fregoli Syndrome during their first psychotic episode compared with those in the primary psychosis group. In addition, patients in the secondary psychosis group were reported to be older. These findings indicate the importance of investigating underlying neurological causes, particularly in individuals who exhibit Fregoli symptoms at an advanced age or during a first psychotic episode.

From a diagnostic perspective, there is no single laboratory test or imaging method that can definitively confirm Fregoli Syndrome. Therefore, a comprehensive psychiatric, neurological, and cognitive evaluation is important. Brain imaging methods may be used when necessary. In terms of treatment, there is no single definitive standard for Fregoli Syndrome; the approach is determined according to the underlying cause and coexisting conditions. Antipsychotic medications may be used to control psychotic symptoms, while different treatment approaches may be considered in some treatment-resistant cases.

In conclusion, Fregoli Syndrome is a rare delusion of misidentification in which an individual perceives different people as different appearances of the same familiar person. It may be associated with psychiatric disorders such as schizophrenia, as well as neurological conditions such as stroke and traumatic brain injury. Findings involving the right hemisphere and frontal regions particularly highlight the neurological aspect of the syndrome. Therefore, Fregoli Syndrome should not be considered solely as a psychiatric symptom; when necessary, underlying neurological and cognitive causes should also be investigated.

References:

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